

Colon & Colorectal Cancer — Clinical Trial & Treatment Timeline

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Clinical Summary

Recent developments in colorectal cancer research highlight significant advancements for patients with Stage IV and advanced disease. Notably, the FDA has approved Pfizer's Braftovi combination therapy for first-line treatment of metastatic colorectal cancer with BRAF mutations, signaling a new standard of care for this patient population. Additionally, several clinical trials are underway exploring innovative treatment combinations, such as dual-target CAR-NK cells and Iparomlimab with chemotherapy, which may offer new options for patients with specific biomarkers or unresectable tumors. Moving forward, it will be important to monitor the outcomes of these trials and their potential impact on treatment protocols for advanced colorectal cancer.

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Situation Summary

Events Detected: 17
High Significance: 2
Top Category: New Trial
Primary Stage: Stage IV
Primary Target: General
Research Activity: Active Monitoring

Event Timeline

EVENT CLUSTER — Safety and Efficacy of Tegavivint in Patients With Metastatic Colorectal Carcinoma

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 11

Mar 11 17:00 · immediate · clinicaltrials.gov

A new clinical trial is investigating the safety and effectiveness of Tegavivint for patients with Stage IV metastatic colorectal cancer, potentially offering a new treatment option for those with advanced disease.

1 source(s) reporting · cluster 1 of 12

EVENT CLUSTER — Dual-Target CAR-NK Cells for Biomarker-Selected Advanced Colorectal Cancer

MEDIUM

STAGE IV

PD-1/PD-L1

NEW TRIAL

CORROBORATED

PUBLISHED MAR 10

Mar 10 18:35 · immediate · [clinicaltrials.gov](#), [clinicaltrials.gov](#), [clinicaltrials.gov](#)

A new clinical trial is exploring the use of dual-target CAR-NK cells in patients with advanced colorectal cancer, potentially offering a promising treatment option for those whose tumors express specific biomarkers.

3 source(s) reporting · cluster 2 of 12

EVENT CLUSTER — A Clinical Study of Iparomlimab and Tuvonralimab Combined With Bevacizumab and Alternating Triweekly CAPOX/mCAPIRI Regimen as First-line Treatment for Unresectable Advanced Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

CORROBORATED

PUBLISHED MAR 3

Mar 10 18:32 · immediate · [clinicaltrials.gov](#), [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of Iparomlimab and Tuvonralimab with Bevacizumab and a specific chemotherapy regimen as a first-line treatment for patients with unresectable advanced colorectal cancer, potentially offering a new option for improved outcomes.

2 source(s) reporting · cluster 3 of 12

EVENT CLUSTER — FOLFOX Chemotherapy Combined With Fruquintinib and Serplulimab as First-Line Conversion Therapy for Initially Unresectable pMMR/MSS Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 3

Mar 10 18:32 · immediate · [clinicaltrials.gov](#), [clinicaltrials.gov](#)

A new clinical trial is exploring the combination of FOLFOX chemotherapy with Fruquintinib and Serplulimab as a promising first-line treatment option for patients with initially unresectable stage IV colorectal cancer, potentially improving their chances for surgical conversion.

2 source(s) reporting · cluster 4 of 12

EVENT CLUSTER — FDA Approves Pfizer's Braftovi Combination for First-Line Metastatic Colorectal Cancer

HIGH

STAGE IV

BRAF

TARGETED THERAPY

CORROBORATED

PUBLISHED FEB 25

Mar 10 18:26 · immediate · [Contract Pharma](#), [Oncology News Central](#)

The FDA has approved Pfizer's Braftovi combination therapy for first-line treatment of Stage IV metastatic colorectal cancer with BRAF mutations, offering new hope for improved outcomes in this challenging stage of the disease.

2 source(s) reporting · cluster 5 of 12

EVENT CLUSTER — A Phase II/III Clinical Trial of RC148 Plus Chemotherapy as 1L Therapy for Unresectable or Metastatic Colorectal Cancer

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 10

Mar 10 18:26 · immediate · clinicaltrials.gov

A new clinical trial is testing the combination of RC148 and chemotherapy as a first-line treatment for patients with unresectable or metastatic colorectal cancer, potentially offering a new option for improving outcomes in this advanced stage of the disease.

1 source(s) reporting · cluster 6 of 12

EVENT CLUSTER — Adding Encorafenib and Cetuximab to FOLFIRI Shows Benefit in BRAF-Mutant Colorectal Cancer

MEDIUM

ALL STAGES

BRAF

TRIAL RESULTS

VERIFIED

PUBLISHED FEB 25

Mar 9 21:21 · immediate · [The ASCO Post](https://www.asco.org)

Combining Encorafenib and Cetuximab with the chemotherapy regimen FOLFIRI has been shown to improve treatment outcomes for patients with BRAF-mutant colorectal cancer.

1 source(s) reporting · cluster 7 of 12

EVENT CLUSTER — Low-Dose Radiotherapy to Sensitize Pucotenlimab Plus CAPEOX for pMMR Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 4

Mar 9 21:21 · immediate · clinicaltrials.gov

A new clinical trial is exploring the use of low-dose radiotherapy to enhance the effectiveness of the drug pucotenlimab combined with CAPEOX chemotherapy for patients with locally advanced rectal cancer, potentially improving treatment outcomes.

1 source(s) reporting · cluster 8 of 12

EVENT CLUSTER — Everolimus in CDK12-Deficient Metastatic Colorectal Cancer (EVER-RECODE)

MEDIUM

STAGE IV

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED FEB 27

Mar 9 21:21 · immediate · clinicaltrials.gov

The EVER-RECODE trial is exploring the use of Everolimus for treating Stage IV metastatic colorectal cancer with CDK12 deficiency, potentially offering a new targeted treatment option for patients with this specific genetic profile.

1 source(s) reporting · cluster 9 of 12

EVENT CLUSTER — Effect of Perioperative Lidocaine or High-Dose Dexamethasone on Immune Response in Colon Cancer Surgery (PILDI Study)

MEDIUM

STAGE III

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 6

Mar 9 21:21 · immediate · clinicaltrials.gov

The PILDI Study is investigating whether administering perioperative lidocaine or high-dose dexamethasone during colon cancer surgery can enhance the immune response, potentially improving outcomes for patients with Stage III colorectal cancer.

1 source(s) reporting · cluster 10 of 12

EVENT CLUSTER — Neoadjuvant Chemoradiotherapy Plus Tislelizumab With or Without Probio-M9 in pMMR/MSS Locally Advanced Rectal Cancer

MEDIUM

STAGE III

GENERAL

NEW TRIAL

CORROBORATED

PUBLISHED MAR 9

Mar 9 21:21 · immediate · clinicaltrials.gov

A new clinical trial is exploring the combination of neoadjuvant chemoradiotherapy with the immunotherapy drug Tislelizumab, potentially enhancing treatment options for patients with locally advanced rectal cancer.

1 source(s) reporting · cluster 11 of 12

EVENT CLUSTER — FOLFIRI and Bevacizumab With or Without Pelareorep for Second-Line Treatment of Metastatic RAS-Mutated, Microsatellite-Stable Colorectal Cancer

MEDIUM

STAGE IV

VEGF

NEW TRIAL

CORROBORATED

PUBLISHED MAR 4

Mar 9 21:21 · immediate · clinicaltrials.gov

A new clinical trial is exploring whether adding Pelareorep to the standard treatment of FOLFIRI and Bevacizumab can improve outcomes for patients with Stage IV metastatic RAS-mutated, microsatellite-stable colorectal cancer.

1 source(s) reporting · cluster 12 of 12

Appendix A — Patient-Friendly Summary

Plain-language overview for patients, caregivers, and family members

What's happening

Recently, there have been several important developments in colon and colorectal cancer research. New clinical trials are opening up that explore different treatment options for patients with advanced disease. One trial is looking at a drug called Tegavivint for Stage IV metastatic colorectal cancer. Another trial is testing a new approach using CAR-NK cells, which are a type of immune cell, to target specific features of tumors. Additionally, the FDA has approved a new combination therapy from Pfizer for patients with BRAF mutations, which could improve outcomes for those with this specific type of cancer. These advancements offer hope for new treatment options that could make a difference in patients' lives.

What this means for you

For patients and their families, these developments mean there may be new paths to consider. If you or a loved one is facing advanced colorectal cancer, it could be worthwhile to ask your oncologist about these new trials and treatments. Questions might include whether you qualify for any of the new clinical trials, what the potential benefits are, and how these options compare to current treatments. Staying informed and engaged in your care is important. While the journey can be challenging, these advancements show that researchers are working hard to find better solutions for patients. Remember, you are not alone in this fight, and there are people dedicated to helping you.

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This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B — Colon & Colorectal Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year · ~52,000 deaths/year · 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s · Adults under 55 now account for ~20% of all new CRC diagnoses · Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG — Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years · FIT (fecal immunochemical test) annually · Cologuard every 1-3 years · CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative — whichever comes first · Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia → immediate evaluation regardless of age

Key Biomarkers — Test All mCRC at Diagnosis

RAS (KRAS/NRAS) · BRAF V600E · MSI/MMR status · HER2 · NTRK fusion · TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) → sotorasib + cetuximab · adagrasib + cetuximab

BRAF V600E (~8-10%) → encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) → pembrolizumab (1st line preferred) · nivolumab ± ipilimumab

HER2 amplified (~2-3%) → trastuzumab + pertuzumab · trastuzumab deruxtecan

RAS/BRAF WT → FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov · NCCN Guidelines (nccn.org) · Colorectal Cancer Alliance (ccalliance.org) · Fight Colorectal Cancer (fightcrc.org) · ACS (cancer.org)

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